

An Older Autistic Adult in Your Care Home / Nursing Team

For care-home, nursing-home & community-nursing staff — supporting an older autistic adult (roughly 60+), diagnosed or not.

The one idea

Autism, through **monotropism**, means an older adult's attention has always locked into a few deep, intense "tunnels." This is the **least-researched part of the autistic lifespan**, and care settings are often the **hardest environments** — sensory-hostile, routine-disrupting, full of unpredictable touch and demand. Many older autistic residents were never identified. Protecting lifelong routines, interests and objects; lowering the load; communicating literally; and carefully separating autistic difference from dementia are the core of good, dignified care.

What you may see — and what's really going on

You may see...	What's often happening
Avoids care, "doesn't like coming in," withdraws	Lifelong negative experiences + sensory/communication barriers — not non-compliance
Distress at routine, room or staff changes	Predictability and possessions are identity and safety anchors
Doesn't report pain; late presentation	Interoception differences; "not complaining" ≠ "not suffering"
Slow, unusual responses; seems confused	Often processing time + lifelong autistic difference — not necessarily dementia
Exhaustion, loss of skills	Possibly decades of accumulated burnout

Try this

- **Build care around predictability:** consistent staff, same sequence, advance warning of every change.
- **Protect lifelong routines, interests and objects** — removing them "for safety" can cause real harm.
- **Investigate pain and physical causes first** for any new distress or decline.
- **Communicate clearly, literally and slowly**, in writing where it helps; allow ample processing time; use supported decision-making.
- **Lower the sensory load** and account for hearing/vision changes compounding lifelong differences.

Avoid this

- Writing off lifelong differences as "just ageing," or burnout as depression.
- Disrupting routines or removing meaningful objects in care.
- Assuming cognitive incapacity from a slow or atypical response — assess, don't assume.

When to get more support

The **dementia-or-autistic-difference?** question is genuinely hard and under-researched — don't assume; seek **autism-informed** assessment (standard screens can misread autistic traits). Watch for **autistic burnout** (decades in the making) and **social isolation** (a major risk). Be transparent about the thin evidence in old age.

If the person is a woman

Older autistic women are an especially under-identified group, having masked for a lifetime; a history of “anxiety,” misdiagnoses or burnout alongside lifelong difference warrants serious consideration of autism.

About this guide

AI-assisted, derived from this project's research drafts — the **Family guide** (Part 5, care homes & hospitals), the **Lifespan Practitioner & Health-Care guide** (Parts 3.5 & 4) and the **Monotropism** brief. Uses identity-first language. **Informational — not medical advice or a diagnostic tool.** Old-age evidence is thin; be honest about uncertainty.

Key sources. Murray, Lesser & Lawson (2005); Dwyer (2025); Klein et al. (2024, aging & autism); Raymaker et al. (2020); Kelly Mahler (2024); OAR (aging on the spectrum); NTG (autism & dementia). Full citations are in the source drafts.